North West Genomic Laboratory Hub (Manchester Site)
Manchester Centre for Genomic Medicine
6th Floor, St Mary's Hospital, Manchester, M13 9WL
Scientific Operational Director: Dr E. Howard
https://mft.nhs.uk/nwglh/ mft.genomics@nhs.net Tel +44(0) 161 276 6122

NHS North West
NHS Genomic Laboratory Hub

Forename:	**TEST**	Report No:	**R24-12AZ-1**
Surname:	**TEST**	Hospital No:	**07994143**
Sex:	**Unknown**	Other No:	**Not provided**
DoB:	**21/03/1999**	Sample:	**DNA**
NHS No:			
Hive Order ID:	**1016299598**		

Collected:	**Not provided**	Received:	**28/07/2024 13:31**	Activated:	**29/07/2024**
				Reported:	**29/07/2024**

Referred by: HIVE UNKNOWN_HIVE, Unknown

Genomics Laboratory Report

Reason for Testing
iGene > Beaker Outbound Message Test 1

RESULT SUMMARY:
Genetic cause not identified.

Result and Interpretation
Whole genome sequence analysis did not identify any pathogenic variant(s) that explain the cause of this individual's clinical presentation. This result does not exclude a genetic diagnosis.

[REASON FOR TESTING - COPY BELOW TEXT TO 'REPORTING REFERRAL INFORMATION TEXT' BOX THEN DELETE]
To investigate the cause of this individual's phenotype (see Appendix I overleaf).
Clinical Indication: [insert Clinical Indication name] (RXXX). The details of any additional panels applied can be found in Appendix I overleaf.

Reported By:
Jonathan Edgerley
Clinical Scientist

Authorised By:

Jonathan Edgerley
Clinical Scientist

APPENDIX I - Test Methodology:

Clinical Indication: *Hypotonic infant (R69.5)*

Affiliated panel(s): *Copy panels here with version numbers*

Whole genome sequencing (Illumina) and analysis of variants prioritised by the Genomics England bioinformatics pipeline for the Genomic Medicine Service gene panel(s) listed above. Please note that the sensitivity of this test is limited by the types of detectable variants, regions of low read depth coverage and incomplete ascertainment of disease-gene associations. Variants are evaluated in accordance with ACMG/ACGS guidance; only pathogenic, likely pathogenic and variants of uncertain significance considered to be clinically actionable are reported.

Patient phenotype: *HPO term 1; HPO term 2 (copy from Congenica HPO terms tab) OR Unaffected parent*

Penetrance: *Complete OR Incomplete*